DPP DEPARTMENTAL POLICY AND PROCEDURE

TITLE:	TRANSFER OF PATIENT FROM ICU TO WARD
DEPARTMENT:	INTENSIVE CARE UNIT
APPLIES TO:	ALL MEDICAL AND NURSING STAFF

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1. PURPOSE

1.1. To provide procedural guidelines on how to transfer patient from ICU to the ward.

1.2. To maintain the continuity of care during transfer from ICU to the ward.

1.3. To help the ward staff in organizing the unit according to priority.

2. DEFINITION & ABBREVIATION

2.1. **Transfer of patient from ICU to ward** – refers to transport of patient to the ward when critical care management is no longer indicated to patient condition.

2.2. **ICU** – Intensive Care Unit

2.3. **NOD** – Nurse on Duty

2.4. **HIS** – Hospital Information System (**MCC** – Medica Cloud Care System)

3. RESPONSIBILITY

3.1. **ICU Physician** – Responsible in providing a written order for the patient to transfer to the ward with clear and complete plan of care.

3.2. **ICU Nurse** – Responsible in shifting the patient from ICU to ward. Informs the ward nurse about the condition of the patient, the equipment and/or machines needed by the patient in the ward

3.3. **Ward Nurse** – Prepares the necessary equipment and/ or machines needed by the patient prior to receiving the patient in the unit.

4. POLICY

4.1. There should be a written order from the doctor before shifting the patient to the wards.

4.2. A clear plan of care should be written by the doctor and transcript by the nurse prior to transfer of patient to the ward

4.3. The ICU doctor through social worker should inform the patient and relatives about the transfer thru designated office.

4.4. Must inform ward nurse on duty with all details about patient's condition for the availability of the bed.

4.5. Ward NOD organizes and prepares the equipment according to patient's need.

4.6. The patient should be transferred out to another area accompanied by an ICU nurse

4.7. Patient conforms to discharge criteria and with written discharge order will be shifted to the ward with or without relative under the responsibility of the staff of the receiving unit.

5. PROCEDURE

5.1. Physician assesses the patient prior in shifting to the lower acuity level.

5.2. Inform the ward NOD with regards to the following:

5.2.1. Patient's data

5.2.2. Patient's diagnosis

5.2.3. Patient's present condition

5.2.4. Equipment needed such as air mattress, oxygen therapy or wheelchair

5.2.5. Infectious Precautions

5.2.6. Presence of Watcher

5.3. Take the possible room number where the patient to be placed.

5.4. Inform patient's relative that patient will be shifted to the ward thru social worker (Morning Shift) and Mudir Munaob (Afternoon and Night Shift);

5.5. Check patients file, review documentation and fill-up Transfer Form

5.6. Verify with the ward charge nurse about the time of transfer.

5.7. Give due medications until the patient leaves the unit.

5.8. Check the patency of intravenous cannula.

5.9. Transport patient to specified ward.

5.9.1. Give complete and accurate endorsement to the ward nurse including bedside, progress notes in MCC and patient's file endorsement.

5.9.1.1. Patient's data

5.9.1.2. Patient's attending physician

5.9.1.3. History of illness; if referral from other hospital, state the name of the hospital.

5.9.1.4. Medications, treatments, Intra-venous fluids

5.9.1.5. Latest vital signs

5.9.1.6. Special procedures done as CT scan ultrasound and others.

5.9.1.7. Whether intubation and mechanical ventilation was done.

5.9.1.8. With or without oxygen inhalations

5.9.1.9. Present diet

5.9.1.10. Skin condition

5.9.1.11. Post-operative patient

5.9.1.11.1. Operation undergone date and time

5.9.1.11.2. Operative site

5.9.1.11.3. Wound dressing

5.9.1.12. Activity in the ward

5.9.1.12.1. Complete bed rest

5.9.1.12.2. Toilet privilege

5.10. Clean and prepare the bed and bedside according to infection control policy as soon as possible for next admission;

5.10. 1. Remove the soiled linens.

5.10. 2. Discard all the things in the bedside table.

5.10. 3. Clean the medical equipment's.

5.10. 4. Inform the cleaners to clean the bed and non-medical equipment's.

5.11. Transfer the patient in the MCC of the receiving unit and endorse to the receiving nurse necessary details of the transfer.

5.12. Record in the discharge bed management logbook and inform and/or sent the logbook to the supervisor.

5.13. Record in the Unit Admission/Discharge logbook the date, time and the ward where the patient was transferred.

6. MEASUREMENT

6.1. 100% of the medical and nursing staff are well oriented and adheres to this policy.

7. REFERENCES /CROSS- REFERENCES

7.1. MOH - Manual of Nursing Policies and Procedures 2nd edition 2011

7.2. Central Committee of NPP 2010 general Directorate of Nursing - MOH.KSA

7.3. APP-QPS-0003 Effective Communication (Handover Process)

8. ATTACHMENT

8.1. ICU Nurses Internal Transfer / Discharge Form during downtime

8.2. Transfer Summary Report (Critical Unit to Lower Acuity)

9. REVISION CLAUSE

9.1. Changes to the hospital policy and procedure template based from the Al-Baha Health Cluster logo, font face and design

9.2. Changes of policy code from DPP-ICU-0019 to DPP-ICU-0012.

9.3. Modification in procedure contents.